

Socioeconomic Contradictions of Child Obesity in India

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Introduction

For decades, child nutrition in India has been overwhelmingly framed through the lens of deprivation. Policy debates, academic research, and public health interventions have historically prioritized the reduction of undernutrition, particularly stunting, wasting, and micronutrient deficiencies among children under five years of age (Menon et al., 2015). This emphasis was shaped by the scale of the problem, where visible forms of hunger and growth failure demanded urgent attention. As a result, nutrition has largely been understood as an issue of insufficient intake, with success measured through improvements in these indicators. India continues to account for a disproportionately large share of the global burden of child undernutrition, and persistent socioeconomic inequalities have sustained high levels of nutritional deprivation across regions and social groups (Nie et al., 2019). National programmes such as the Integrated Child Development Services (ICDS), the Mid-Day Meal Scheme, and POSHAN Abhiyaan reflect this long-standing policy orientation toward caloric insufficiency and growth faltering.

However, alongside these entrenched challenges, a less anticipated nutritional concern has begun to emerge: the rise of overweight children and obesity (Prusty et al., 2022). Although historically considered marginal in low-income settings, excess weight among young children is increasingly being documented in India. This shift challenges long-standing assumptions that obesity is largely irrelevant in contexts characterized by widespread poverty and undernutrition.

Child obesity in India is emerging within a context where undernutrition remains widespread (D'souza & Ramesh, 2024). Unlike high-income countries, where obesity typically follows the decline of basic nutritional deficiencies, India is experiencing overlapping forms of malnutrition where persistent stunting, anemia, and underweight coexist with rising levels of child overweight.

Understanding this paradox requires situating child obesity within the broader process of nutrition transition. This theory highlights a shift from traditional diets toward processed, energy-dense foods, accompanied by reduced physical activity and increasingly sedentary lifestyles.

Existing research suggests that the distribution of child overweight in India is highly uneven across regions. Prevalence varies widely across states and local contexts, reflecting differences in socioeconomic conditions, food environments, and infrastructural development. Such spatial variation highlights the importance of examining child obesity beyond national averages.

Urban areas have traditionally been associated with higher obesity risks due to sedentary lifestyle. However, emerging scholarship suggests that obesogenic environments¹ are no longer confined to cities alone. The spread of such environments across diverse settings challenges earlier assumptions about rural contexts serving as a protective buffer against obesity (Lake & Townshend, 2006).

Crucially, childhood obesity in India is not restricted to affluent households. Increasing evidence indicates that excess weight increasingly affects children from middle- and lower-income families, including the urban poor. This challenges the view of obesity as a consequence of affluence and instead points to structural conditions in which inexpensive, calorie-dense but nutrient-poor foods are widely accessible, while nutritious diets remain limited (UNICEF, 2024). In such contexts, children may consume sufficient or excess calories while remaining deficient in essential micronutrients, thereby experiencing obesity alongside anemia and other forms of deprivation.

Early childhood obesity warrants particular concern because it increases the risk of non-communicable diseases (NCDs) such as diabetes and cardiovascular disease in adulthood, especially when combined with early-life undernutrition. Thus, rising child obesity is not merely an isolated nutritional anomaly but a precursor to a future public health burden that may exacerbate existing health inequalities.

This chapter argues that childhood obesity in India should be understood not simply as a consequence of affluence, but as a structural outcome of uneven development. In contexts where dietary change, urbanization, and market integration occur without corresponding improvements in nutritional security and living conditions, caloric excess can coexist with persistent deprivation. Childhood obesity thus reflects deeper socioeconomic contradictions embedded within India's ongoing nutrition transition.

Despite these risks, child obesity remains marginal within India's nutrition policy framework. Current interventions continue to prioritize undernutrition, often overlooking the emerging challenges

¹ *The obesogenicity of an environment has been defined as 'the sum of influences that the surroundings, opportunities, or conditions of life have on promoting obesity in individuals or populations*

associated with excess weight (Kim et al., 2017). This policy blind spot reflects a deeper conceptual issue: the difficulty of reconciling obesity with deprivation. As a result, child obesity is frequently treated as an exception or an urban elite problem rather than as a structural outcome of India's changing food systems, living environments, and socioeconomic inequalities.

To address these limitations, this chapter makes a clearer argument: childhood obesity in India is best understood as a result of structural inequality in a context of uneven development. It is not simply a move away from deprivation. Instead, rising obesity reflects a shift in how disadvantage shows up. Children may have enough calories, but still face poor diet quality, stressful environments, and difficult living conditions.

This view moves the focus away from individual choices or income-based explanations. It brings attention to the wider social, economic, and environmental factors that shape nutrition. Looking at the evidence through this lens, the chapter shows that the coexistence of undernutrition and obesity is not a contradiction, but a predictable outcome of India's uneven nutrition transition.

Conceptual Framework: Uneven Nutrition Transition

The concept of nutrition transition provides a useful framework to understand shifts in dietary patterns and physical activity associated with economic development, urbanization, and globalization. In many high-income countries, this transition has followed a relatively linear trajectory, with declines in undernutrition followed by rising prevalence of overweight and obesity (Popkin et al., 2020; FAO et al., 2023). In high-income countries, as incomes rise and food systems and public health improve, undernutrition declines and is later replaced by rising overweight and obesity. But this pattern does not fit to India.

However, in low- and middle-income countries such as India, this process is neither uniform nor sequential. Instead, it is characterized by an uneven and compressed transition, where rising obesity coexists with persistent undernutrition (WHO, 2022). Here, different stages of the transition exist at the same time. Instead of moving clearly from scarcity to excess, people face overlapping nutritional

risks. These are shaped by uneven access to food, services, and basic infrastructure. In this setting, obesity is not simply a sign of abundance. It is shaped by structural constraints and limited choices.

This unevenness is closely linked to broader patterns of socioeconomic inequality. Economic growth in India has been spatially and socially differentiated, resulting in unequal access to nutritious food, healthcare, sanitation, and safe environments for physical activity (IIPS & MoHFW, 2021). As a result, dietary change often occurs under conditions of constraint rather than abundance, where populations experience shifts toward calorie-dense diets without corresponding improvements in nutritional quality (Baker et al., 2020).

The said transformations in food systems and living environments further reinforce these patterns. The increasing availability of inexpensive, energy-dense, and ultra-processed foods, driven by market expansion and globalization, has significantly altered consumption patterns across both urban and rural settings (Baker et al., 2020). At the same time, urbanization and changes in work, transport, and built environments have reduced opportunities for physical activity (Sharma & Sachdeva, 2023). Sedentary lifestyles, combined with environmental constraints such as limited access to safe recreational spaces, contribute to declining energy expenditure (UNICEF, 2021). These shifts reflect broader structural conditions rather than individual choices alone (insert citation if present).

The coexistence of obesity and deprivation can be understood through multiple interacting pathways. Diets dominated by calorie-dense but nutrient-poor foods can lead to excess energy intake alongside micronutrient deficiencies (WHO, 2022). Early-life undernutrition may increase susceptibility to obesity later in life through metabolic adaptations (Victora et al., 2021). In addition, chronic stress and adverse living conditions associated with poverty can influence physiological processes linked to weight gain (Hamilton & Steptoe, 2023; Selvakumaran et al., 2023). Together, these pathways suggest that obesity in such contexts reflects nutritional imbalance shaped by structural conditions rather than simple excess (insert citations here).

Overall, this framework shows that childhood obesity in India is shaped by changes in diet, living conditions, and social inequality, all within a context of uneven development. It also helps explain how biological, environmental, and social factors work together to influence health outcomes. The next section builds on this by examining how these patterns appear across different regions and population groups in India.

Patterns of Childhood Obesity in India

This section examines patterns of childhood obesity in India using data from the National Family Health Survey (NFHS). The focus is not only on describing trends, but also on understanding how obesity is distributed across regions, socioeconomic groups, and demographic categories, within a context of uneven development.

In India, most national level evidence on child nutrition comes from the National Family Health Survey (NFHS). It is the most comprehensive and regularly conducted source of demographic and health data (IIPS & MoFHW, 2021). The survey is carried out by the International Institute of Population Sciences (IIPS) under the guidance of the Ministry of Health and Family Welfare and provides nationally representative information on maternal and child health, nutrition, fertility and household living conditions. Since the early 1990s, successive rounds of NFHS have made it possible to track changes in nutritional outcomes among children under five years of age across states and social groups in India (Khadse & Shaikh, 2025).

In line with this, a comparison between NFHS-4 and NFHS-5 shows a gradual increase in the prevalence of overweight among children under five, rising from 2.1% to 3.4%. Although the overall prevalence remains lower than that of undernutrition indicators, the consistent increase across survey rounds suggests a shift in the distribution of child overweight. Though the difference between the two rounds of survey data is minimal, if it is contrasted against the vast population of children in India, it would make a significant difference. Given the large child population of India, even a small percentage change translates into a significant number of affected children.

Overall, NFHS data indicate that childhood overweight remains less prevalent than other forms of malnutrition but shows an increase across successive survey rounds.

Going beyond national trends and averages, there are significant variations at the state level with respect to the level of child overweight in the country.

Figure 1 and 2 presents the prevalence of overweight among children across Indian states. A clear regional variation is observed, with relatively higher prevalence in more urbanized and economically advanced states, while lower levels are seen in less developed regions. However, this pattern is not

uniform. The presence of overweight across a wide range of states, including those with lower average incomes, suggests that childhood obesity is not confined to affluence alone.

 *Source: nhm.gov.in & mohfw.gov.in*

Notable increases are observed in the northeastern part of the country, besides the western and southern parts of the country. There are several states like Sikkim, Mizoram, Tripura, Maharashtra, Punjab, and West Bengal showing increased levels of child overweight in the NFHS-5 survey. On the contrary, there are quite small changes observed in quite a number of other states, with exceptions in the case of Goa and Tamil Nadu, where it has declined.

 *Source: nhm.gov.in & mohfw.gov.in*

A comparable trend is seen at the level of Union Territories. UTs like Ladakh, Jammu and Kashmir, Lakshadweep, and Andaman and Nicobar Islands have marked increases in overweight children, while others have seen little change, except in the cases of Dadra and Nagar Haveli, and Daman and Diu.

This distribution reflects the uneven nature of India's nutrition transition. While some regions show more advanced dietary changes, others continue to face high levels of undernutrition alongside rising obesity. This means that the patterns do not represent a simple shift from one form of malnutrition to another, but the coexistence of multiple burdens. These differences are shaped not just by income levels, but also by how regions are connected to changing food systems, levels of urbanization, and access to infrastructure and services. As a result, higher obesity in some states may reflect more advanced transitions, while its presence in less developed regions suggests that these changes are happening under conditions of constraint rather than prosperity.

In addition to this, figure 3 and 4 compares the prevalence of childhood overweight across urban and rural areas. As expected, higher levels are generally observed in urban settings, reflecting differences in lifestyle, dietary patterns, and access to processed foods. However, the gap between urban and rural areas is narrower than often assumed.

 *Source: nhm.gov.in & mohfw.gov.in*

The record in urban areas is relatively and notably higher in states such as Sikkim, Mizoram, Tripura, Tamil Nadu, Telangana, and West Bengal. In contrast, Bihar, Assam, and Jharkhand show smaller urban-rural gaps.

 *Source: nhm.gov.in & mohfw.gov.in*

The contrasts between UTs, on a larger scale, are sharper and of significance, where Ladakh, Lakshadweep, Jammu and Kashmir, and Andaman and Nicobar Islands also display very high levels of overweight children, particularly in urban settings, though in rural, they also remain high. Then there are UTs of Chandigarh and Puducherry, which keep displaying disparities, though to a smaller extent.

The presence of overweight in rural areas shows that the drivers of obesity are no longer limited to cities. With the spread of markets, changes in food systems, and shifts in work patterns, obesogenic conditions are now reaching rural populations as well. This has reduced the gap between urban and rural areas and challenges the idea that rural settings are protected from obesity. Instead, it suggests that the nutrition transition is spreading across regions, though under different constraints.

Taken together, these patterns indicate that childhood obesity in India is distributed across regions and socioeconomic groups in ways that reflect broader structural inequalities. Rather than being concentrated in a single segment of the population, obesity is emerging across diverse contexts, including those still affected by undernutrition.

This coexistence highlights the limitations of viewing malnutrition through a singular lens. The patterns observed suggest that obesity and undernutrition are not separate phenomena, but interconnected outcomes of the same processes of uneven development and dietary transition.

Overall, the empirical evidence demonstrates that childhood obesity in India is not a uniform or isolated trend, but part of a broader and more complex transformation in the country's nutritional landscape. The presence of overweight across regions, income groups, and settlement types underscores the need to move beyond simplified explanations based on affluence. The following section builds on these findings to examine the structural drivers underlying these patterns.

Structural Drivers of Childhood Obesity in India

The patterns observed in the previous section show that childhood obesity in India cannot be explained by a single factor. Instead, it is shaped by interconnected changes in food systems, living conditions, and socioeconomic inequalities. These factors influence both what children eat and how they live. Limited access to nutritious food, changing work and lifestyle patterns, and differences in income and infrastructure together shape daily habits and health outcomes, showing how obesity develops within contexts of inequality rather than from individual behaviour alone.

Food systems Transformations

One of the most important changes behind these patterns is the transformation of food systems. Over the past few decades, inexpensive, energy-dense, and ultra-processed foods have become widely available in both urban and rural areas (Sawyer et al., 2021). These foods are often more affordable and easier to access than fresh and nutritious options, especially for low- and middle-income households. As a result, diets are increasingly high in calories but low in nutritional quality, contributing to the rise of obesity along with micronutrient deficiencies. These changes are closely linked to food insecurity, where rising food prices and limited income reduce access to diverse and healthy foods, pushing households toward cheaper, energy-dense options and reinforcing the double burden of malnutrition.

Urbanization and Sedentary Environments

Urban settings further increase these risks by shaping both diet and physical activity. NFHS-5 reports higher levels of obesity in urban areas compared to rural areas, with about 4.2% of the population classified as obese. Reduced physical activity, sedentary lifestyles, work-related stress, and unhealthy dietary practices all contribute to these obesogenic environments (Kirchengast et al., 2021). While wealthier households may have better access to healthy foods, healthcare, and recreational spaces, these options are often limited for the urban poor. As a result, overcrowded living conditions, financial stress, and greater exposure to energy-dense diets increase the risk of obesity among lower-income urban populations (Lunagariya et al., 2019). This shows how urban environments bring together both lifestyle changes and inequality, contributing to the spread of obesity beyond traditionally affluent groups.

Structural Inequality and Living Conditions

Beyond food systems and urban environments, broader structural inequalities also shape the conditions in which obesity develops. These operate through three key pathways: limited living conditions, environmental exposures, and long-term disadvantage. These constraints are closely linked to food insecurity, which limits access to adequate and nutritious diets (Lopes et al., 2023).

Limited access to sanitation remains a major issue for lower socioeconomic groups. Repeated infections, ongoing physiological stress, and poor nutrient absorption affect overall nutritional status and create a cycle of vulnerability. NFHS-5 data show that only 69% of households in India have access to improved sanitation, with education, income, and social marginalisation acting as key barriers (Kumar et al., 2024).

Frequent infections increase metabolic stress and worsen micronutrient deficiencies. Poor nutritional status, especially iron deficiency, can also weaken immune responses, including the effectiveness of vaccines, increasing susceptibility to infections (Abuga et al., 2023). Indoor air pollution further adds to these risks, as many households still rely on solid fuels, leading to continuous exposure to polluted air. This contributes to ongoing stress on the body and increases the risk of metabolic problems, particularly among vulnerable populations (Gupta et al., 2020). Obesity is also linked to low-grade inflammation, which can affect iron use in the body and increase the risk of anemia, further worsening nutritional outcomes (Alshwaiyat et al., 2021; Hamilton et al., 2023).

Social, Regional, and Historical Inequalities

Obesity in India also reflects clear social, regional, and historical differences rather than uniform lifestyle changes. There is increasing evidence that obesity is rising among historically disadvantaged caste groups, especially in urban and peri-urban areas. This is linked to economic constraints, dependence on calorie-dense diets, unstable work conditions, stress, and limited access to healthcare (Thapa et al., 2021). Regional patterns show uneven development, with higher prevalence in southern, northeastern, and western regions, along with a growing burden in rural areas. For example, obesity among women increased from 8.6% in NFHS-3 to 19.7% in NFHS-5, and among men from 7.3% to 19.3% (Singh et al., 2023). These trends reflect the spread of dietary and lifestyle changes beyond urban areas.

Long-term disadvantage also plays an important role. Many communities have experienced undernutrition, poor maternal health, and repeated infections over generations, leading to low birth weight and early-life malnutrition (Pandit et al., 2021). These early conditions can influence body composition, increasing the risk of fat accumulation and metabolic diseases later in life (Yajnik, 2024). Together, these findings show that obesity in disadvantaged populations is closely linked to long-term inequality and intergenerational disadvantage.

Household as the Site of Transmission

Obesity is ultimately shaped by multiple interacting factors, including dietary practices, physical activity patterns, environmental stressors, and economic constraints (Ahmed & Mohammed, 2025). These broader structural influences are expressed within households, where daily routines, caregiving practices, and food environments shape children's health.

The household increasingly reflects changing economic and lifestyle conditions. Time constraints, increased reliance on convenience foods, and rising screen use are reshaping daily habits. Social norms and perceptions of health also influence feeding practices and activity patterns. As a result, the household becomes a key site where structural disadvantage is translated into biological outcomes, reinforcing obesity risk across generations.

Household Modernization and Lifestyle Changes

The structural processes discussed above do not operate in isolation but are expressed through everyday practices within households. The household thus serves as a critical site where broader economic and environmental transformations are translated into daily routines, dietary behaviours, and patterns of physical activity. Building on the structural inequalities discussed in Section 4, this section shifts attention to the household as a key site where modernization translates into changes in child health. While obesity is not confined to elite groups, the pathways through which socioeconomic mobility reshapes daily routines, consumption practices, and caregiving are most visible within urban and peri-urban middle-class households. In this context, modernization reflects not only rising incomes but a reconfiguration of domestic activity, time use, and consumption norms.

Household Assets and the Mechanization of Domestic Life

The acquisition of household assets in India is commonly used as an indicator of socioeconomic mobility, yet such assets often facilitate changes in daily activity patterns that have metabolic consequences. As per Choudhary et al. (2023), a significant correlation between high standard-of-living scores and pediatric obesity, driven largely by the transition to mechanized transport and labor-saving domestic appliances. For children, this means that the incidental physical activity once inherent to the neighborhood or domestic chores has been replaced by a sedentary infrastructure. This shift is particularly visible in first-generation urban families where the removal of manual tasks has not been replaced by a culture of recreational exercise.

Digital Dependency and Compromised Satiety Regulation

A significant factor in the changing household landscape is the ubiquity of digital devices. Beyond the sedentary nature of screen time, Thorat et al. (2025) identified that in urban centers like Pune, excessive screen time (over 2 hours daily) was associated with an adjusted odds ratio (OR) for obesity that was over 35 times higher than that of low-exposure peers. The mechanism involved is "distracted eating," where digital media acts as a sensory competitor that overrides physiological cues of satiety. This dependency suggests that the modern household is becoming a space where consumption is passive and disconnected from bodily hunger signals.

Aspirational Consumption and the Nutrition Knowledge Gap

The nutrition transition is strongly shaped by aspirational consumption and the symbolic value attached to "modern" food products. Bhardwaj et al. (2025) note that the market for ultra-processed foods (UPFs) in India is growing at an annual rate of 13.37%, with a direct correlation ($\beta=0.31$) between UPF intake and higher BMI in children. In many middle-income households, high professional education does not equate to high nutrition literacy. Commercially prepared "health drinks" and fortified snacks are often perceived as superior to traditional diets due to aggressive marketing. This "aspiration gap" leads families to consume calorie-dense, nutrient-poor foods as a performance of socioeconomic status.

Post-Pandemic Behavioral Shifts

The COVID-19 pandemic acted as a permanent catalyst for sedentary household habits. Jha and Mehendale (2022) describe a "lockdown legacy" where the collapse of boundaries between school and home led to permanent shifts in snacking behavior and reduced outdoor mobility. This environment is particularly toxic when combined with findings from Manapurath et al. (2024), which highlight that maternal BMI and household-level sedentary behaviors remain the strongest predictors of child weight-for-age Z-scores in the NFHS-5 dataset.

These household-level changes show how broader economic and social transformations shape everyday practices. They are not independent drivers of obesity, but reflect wider structural conditions. At the same time, their persistence points to gaps in existing nutrition policies and regulatory frameworks. Understanding these links is important for connecting broader changes with individual health outcomes.

Policy Blind Spots and Programmatic Challenges

India's policy response to nutrition has historically focused on addressing hunger and undernutrition. This focus has shaped the design and implementation of major public health programs, with priority given to improving calorie intake and reducing visible forms of deprivation. However, as patterns of malnutrition have changed, this approach has not adapted at the same pace. As a result, existing systems often struggle to respond effectively to the rising burden of obesity, especially in contexts where undernutrition and deprivation still persist. This section examines these limitations and highlights key policy gaps in addressing this changing nutritional landscape.

The Legacy of Hunger-Centric Programming

India's flagship programs, such as the Integrated Child Development Services (ICDS), were designed to address caloric deficits. Siddiqui et al. (2025) argue that while these programs successfully reduced underweight rates from 37% to 32% (2016–2021), they are operationally blind to obesity. Because the "Poshan Tracker" and frontline workers prioritize reaching minimum weight targets, an overweight child is often not flagged for intervention. This reflects a failure to adapt to the "double burden" where under- and over-nutrition co-occur within the same districts.

Regulatory Gaps in the Food Environment

A major challenge lies in the lack of stringent regulation regarding the marketing of high-fat, sugar, and salt (HFSS) products. Singh et al. (2022) conducted a randomized field experiment in six Indian states and found that while "Warning Labels" were the most effective way for consumers to identify unhealthy foods, there has been significant industry resistance to mandatory Front-of-Pack Labeling (FOPL). Without these mandatory regulations, the aspirational consumption patterns discussed in Section 5 remain unchecked by the state.

The "Missing Middle" in Adolescent Health

Current nutrition interventions in India place substantial emphasis on the "First 1,000 Days," resulting in limited monitoring and intervention for school-age children and adolescents. Studies note that school-based programmes often lack integrated components that combine physical activity, nutrition education, and supportive policy environments (Nikooyeh et al., 2025). As children age out of early childhood services, public health systems provide few mechanisms to track metabolic health, creating a critical gap during a period when obesity risk often accelerates.

These gaps reflect a broader limitation in current policy frameworks, which continue to treat undernutrition and obesity as separate issues rather than connected outcomes of the same structural processes. Addressing the double burden of malnutrition therefore requires more integrated approaches that take into account changing food systems, living conditions, and ongoing social inequalities.

Beyond these specific gaps, a deeper issue lies in how nutrition itself is defined and measured within policy frameworks. Most large-scale programmes continue to rely on anthropometric indicators such as weight-for-age or body mass index, which are effective for identifying visible deficits but less suited to capturing diet quality or early metabolic risk (WHO, 2020; UNICEF, 2021). This narrow focus reinforces a binary understanding of malnutrition as either deficiency or excess, without recognising the continuum that links the two. As a result, children who fall outside severe categories often remain invisible within the system, even when they are exposed to diets that increase long-term health risks.

Institutional fragmentation further complicates the policy response. Responsibility for nutrition is spread across multiple ministries and departments, including health, women and child development, education, and food distribution. While this reflects the multi-dimensional nature of the issue, coordination across these sectors remains limited in practice. Policy reviews have repeatedly noted

gaps in convergence under programmes such as POSHAN Abhiyaan, particularly in aligning data systems, targets, and accountability structures (NITI Aayog, 2021; World Bank, 2020). In the absence of strong coordination mechanisms, efforts to address the double burden tend to remain partial and uneven.

Urbanisation introduces another layer of complexity that current policies are only beginning to address. Rapid growth of informal settlements, changing work patterns, and limited access to safe public spaces have altered both dietary and physical activity behaviours. Households increasingly rely on inexpensive, processed, and ready-to-eat foods that are energy-dense but nutritionally poor (FAO et al., 2022; Popkin et al., 2020). Yet, many nutrition programmes continue to be designed around rural models of deprivation, creating a mismatch between programme design and the realities of low-income urban settings.

Another important blind spot is the limited engagement with behavioral and social drivers of diet. While policy frameworks often acknowledge the role of awareness, interventions tend to rely heavily on information campaigns that assume knowledge will translate into better choices. However, evidence shows that food preferences are shaped by affordability, availability, and aggressive marketing of high-fat, sugar, and salt products, particularly among children and adolescents (UNICEF, 2019; Swinburn et al., 2019). Without addressing these structural influences, behavior-focused strategies risk shifting responsibility onto individuals while overlooking the constraints they face.

Data systems also lag behind the changing nature of malnutrition. Large-scale surveys such as the National Family Health Survey provide valuable insights but are conducted periodically and are not designed for real-time monitoring or localized action (IIPS & ICF, 2021). At the same time, frontline workers often face high workloads and limited training in identifying or managing emerging forms of malnutrition such as childhood overweight (Kapur et al., 2020). This affects both early detection and the ability to provide appropriate counselling, pointing to the need for stronger data systems and capacity-building at the last mile.

Conclusion

This chapter has demonstrated that the rise of child obesity in India represents one of the most striking contradictions of the country's contemporary nutrition landscape. Far from replacing undernutrition,

child obesity has emerged alongside persistent stunting, anemia, and other forms of deprivation. The coexistence reflects deeper structural transformations in India's food systems, urban environments, and patterns of socioeconomic inequality. Instead, India exemplifies a context in which nutritional deprivation and excess coexist within the same populations, households, and even individuals, revealing deep structural tensions embedded in processes of economic growth and social change. The chapter has also framed child obesity in India as an emerging and deeply paradoxical outcome of the country's ongoing nutrition transition. Historically, India's child nutrition agenda has been shaped by the urgent challenge of undernutrition, and this focus remains justified given the persistence of stunting, wasting, and micronutrient deficiencies (Arora et al., 2025). However, the gradual rise of child overweight and obesity alongside these long-standing problems signals a fundamental shift in the nature of malnutrition in India. Rather than representing a departure from deprivation, child obesity reflects its transformation under conditions of rapid social, economic, and dietary change.

By situating child obesity within the framework of multiple burdens of malnutrition, this chapter highlights how nutritional excess and deficiency are not mutually exclusive but frequently coexist. Obesity among Indian children often emerges in contexts marked by poor diet quality, inadequate sanitation, environmental risks, and limited opportunities for physical activity. These patterns challenge affluence-based explanations of obesity and underscore the importance of socioeconomic contradictions, where aspirations for modernity intersect with structural constraints and inequality.

The analysis also emphasizes why early childhood obesity demands urgent attention. As a critical life stage, early childhood shapes long-term health trajectories, and excess weight acquired during this period significantly increases the risk of non-communicable diseases later in life. Ignoring obesity in early childhood therefore risks compounding India's future disease burden and undermining investments in child development.

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